

ORAL CANCERS II

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Learning Objectives

- Define neoplasia
- Classify oral neoplasms
- Describe benign and malignant lesions
- Discuss oral cancer
- Understand salivary gland tumors

Definition of Neoplasia

- **Neoplasm:** An abnormal mass of tissue whose growth exceeds and is uncoordinated with normal tissues.

Characteristics:

- Autonomous growth
- Progressive enlargement
- Persists after stimulus removal

Classification of Neoplastic Lesions

Benign Tumors

- Papilloma
- Fibroma
- Hemangioma
- Lymphangioma
- Lipoma
- Pleomorphic adenoma

Malignant Tumors

- Squamous cell carcinoma
- Verrucous carcinoma
- Mucoepidermoid carcinoma
- Adenoid cystic carcinoma
- Malignant melanoma
- Lymphoma

Benign Epithelial Neoplasms

Squamous Papilloma

Clinical Features:

- Exophytic growth
- Cauliflower appearance
- Painless

Common Sites:

- Tongue
- Soft palate
- Uvula



Fibroma

Irritation Fibroma

Clinical Features:

- Smooth nodule
- Firm consistency
- Same color as mucosa

Common Sites:

- Buccal mucosa
- Tongue
- Lip



Lipoma

Benign Tumor of Fat Tissue

Features:

- Soft
- Yellowish
- Slow growing

Common Sites:

- Buccal mucosa
- Vestibule



Hemangioma

- Benign Vascular Tumor

Features:

- Red or bluish lesion
- Blanches on pressure
- Present from childhood



Lymphangioma

Tumor of Lymphatic Vessels

Features:

- Pebbly surface
- Common on tongue
- Causes macroglossia



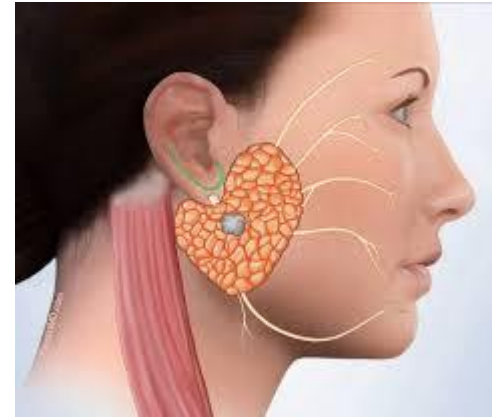
Benign Salivary Gland Tumors

Pleomorphic Adenoma

- Most common salivary gland tumor.

Features:

- Slow-growing
- Painless
- Usually parotid gland



Legend

Fig 1. Intra-oral view of the pathology.



Warthin Tumor

Features:

- Usually parotid
- Associated with smoking
- Slow growing



Oral Potentially Malignant Disorders

- Leukoplakia
- Erythroplakia
- Oral submucous fibrosis
- Lichen planus

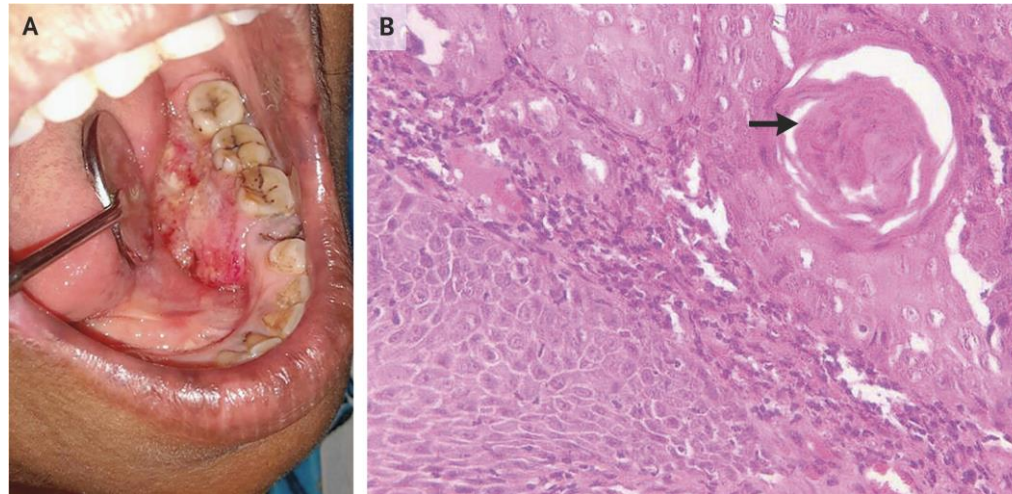


Squamous Cell Carcinoma

- Accounts for >90% of oral cancers.

Common Sites:

- Lateral tongue
- Floor of mouth
- Buccal mucosa



Clinical Features of SCC

- Non-healing ulcer
- Induration
- Pain
- Bleeding
- Difficulty swallowing

TNM Staging

- T = Tumor size
- N = Lymph node involvement
- M = Metastasis

Used for treatment planning.

Verrucous Carcinoma

Features:

- Low-grade SCC variant
- Warty appearance
- Slow growing
- Rare metastasis



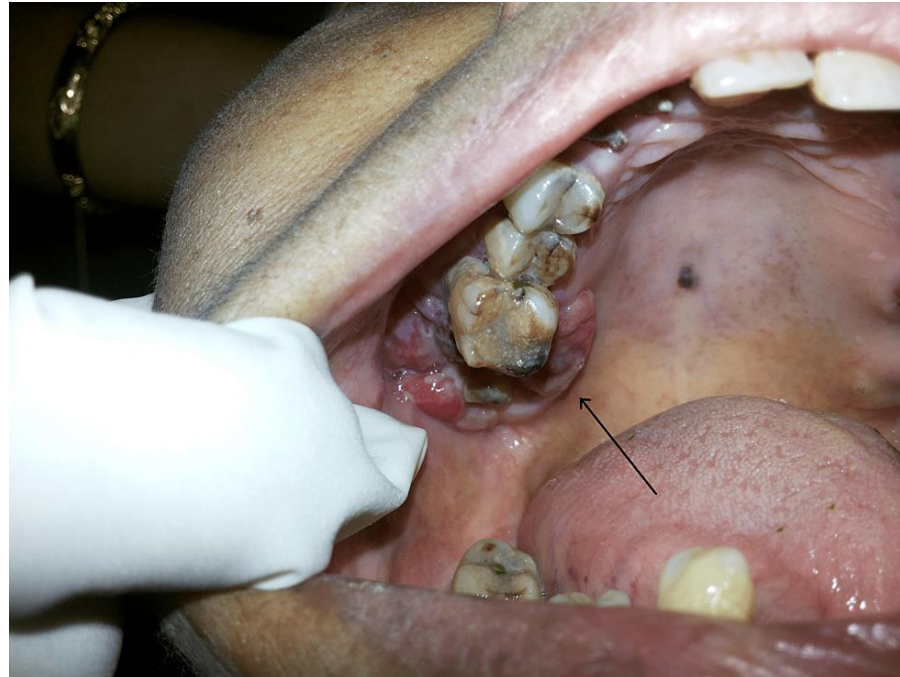
Malignant Melanoma

Origin:

- Melanocytes

Features:

- Dark pigmentation
- Irregular borders
- Rapid progression



Oral Lymphoma

Origin:

- Lymphoid tissue

Common Sites:

- Palate
- Gingiva



Malignant Salivary Gland Tumors

Major Types:

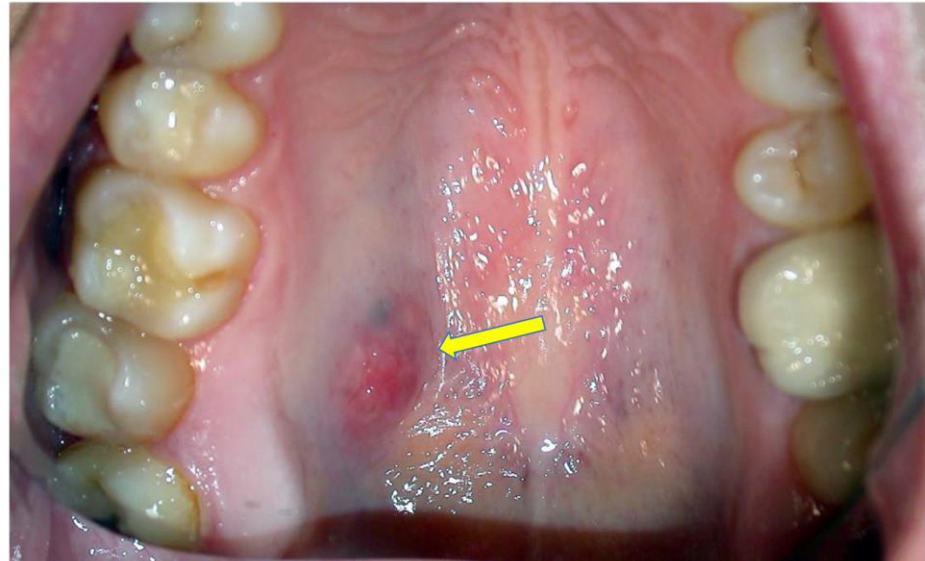
- Mucoepidermoid carcinoma
- Adenoid cystic carcinoma
- Acinic cell carcinoma

Mucoepidermoid Carcinoma

- Most common malignant salivary tumor.

Features:

- Slow enlargement
- May appear bluish
- Variable prognosis



Adenoid Cystic Carcinoma

Features:

- Painful lesion
- Perineural invasion
- Frequent recurrence



Acinic Cell Carcinoma

Features:

- Usually parotid gland
- Low-grade malignancy
- Better prognosis



Diagnosis

- Clinical examination
- Imaging
- FNAC
- Biopsy
- Histopathology

Management

Benign tumors:

- Surgical excision

Malignant tumors:

- Surgery
- Radiotherapy
- Chemotherapy

Summary

- Oral neoplasms may be benign or malignant.
- SCC is the most common oral cancer.
- Pleomorphic adenoma is the most common benign salivary tumor.
- Mucoepidermoid carcinoma is the most common malignant salivary tumor.
- Early diagnosis improves prognosis.